

IN THE SENATE

SENATE BILL NO. 1367

BY STATE AFFAIRS COMMITTEE

AN ACT

RELATING TO PHARMACY BENEFIT MANAGERS; AMENDING CHAPTER 3, TITLE 41, IDAHO CODE, BY THE ADDITION OF A NEW SECTION 41-349A, IDAHO CODE, TO ESTABLISH PROVISIONS REGARDING DISPENSING FEES; AMENDING SECTION 41-349, IDAHO CODE, TO DEFINE TERMS, TO REVISE PROVISIONS REGARDING DISPENSING FEES, AND TO ESTABLISH PROVISIONS REGARDING DUTIES AND RESTRICTIONS PERTAINING TO PHARMACY BENEFIT MANAGERS AND THIRD-PARTY PAYERS; AND DECLARING AN EMERGENCY.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. That Chapter 3, Title 41, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW SECTION, to be known and designated as Section 41-349A, Idaho Code, and to read as follows:

41-349A. DISPENSING FEES. (1) A plan sponsor, pharmacy benefit manager (PBM), or third-party payer shall ensure that reimbursement to independent pharmacies for each drug dispensed is an amount that is not less than the sum of the national average drug acquisition cost (NADAC) as provided for in subsection (2) of this section and the professional dispensing fee as provided for in subsection (3) of the section.

(2) The NADAC shall be the published price in effect for the day that a drug claim is billed by a pharmacy. However, if a particular drug does not have a published NADAC, the reimbursement to an independent pharmacy shall be:

(a) For generic drugs, one hundred percent (100%) of published wholesale acquisition costs; and

(b) For brand name drugs, one hundred percent (100%) of wholesale acquisition costs.

(3) The minimum professional dispensing fee for independent pharmacies shall be twelve dollars and thirty-five cents (\$12.35), subject to an annual increase as provided for in this subsection. On January 1 of each year, every plan sponsor, PBM, and third-party payer shall increase the amount of the minimum professional dispensing fee for independent pharmacies to adjust for inflation. Inflation shall be measured by the annual percentage increase, if any, in the consumer price index for all urban consumers (CPI-U) as published by the United States department of labor, bureau of labor statistics, for all items.

(4) The Idaho department of insurance shall issue a letter and guidance to every plan sponsor, PBM, and third-party payer registered with the state of Idaho no later than thirty (30) days after the effective date of this section. All reimbursement rates shall be in full force and effect as of that date, and plan sponsors, PBMs, and third-party payers shall issue additional payments as needed to independent pharmacies to cover any deficiencies in payment made after the effective date of this section.

(5) The Idaho department of insurance shall issue a letter and guidance to every plan sponsor, PBM, and third-party payer registered with the state of Idaho no later than January 15 of each year. Such letter shall include the updated dispensing fee that will become effective on March 1 of the same calendar year.

(6) Annually, on March 1, plan sponsors, PBMs, and third-party payers shall begin paying the updated dispensing fee as provided for in subsection (5) of this section.

(7) In the event that an issuance date provided for pursuant to this section falls on a weekend or a national holiday, the next business day shall become the required date of issuance.

SECTION 2. That Section 41-349, Idaho Code, be, and the same is hereby amended to read as follows:

41-349. PHARMACY BENEFIT MANAGERS. (1) As used in this section:

(a) "Brand name or generic effective rate" means the contractual rate set forth by a pharmacy benefit manager for the reimbursement of covered brand name or generic drugs, calculated using the total payments in the aggregate, by drug type, during the performance period. The effective rates are typically calculated as a discount from industry benchmarks, such as average wholesale price or wholesale acquisition cost.

(b) "Dispensing fee" means a fee intended to cover reasonable costs associated with providing a drug to a covered person. This cost includes but is not limited to the pharmacist's services and the overhead associated with maintaining the facility and equipment necessary to operate the pharmacy. The dispensing fee shall be set pursuant to the provisions of section 41-349A, Idaho Code.

(c) "Effective rate guarantee" means the minimum ingredient cost reimbursement a pharmacy benefit manager guarantees it will pay for pharmacist services during the applicable measurement period.

(d) "Independent pharmacy" means any pharmacy not owned or affiliated with a pharmacy benefit manager.

~~(d)~~ (e) "Maximum allowable cost" means the maximum amount that a pharmacy benefit manager will reimburse a pharmacy for the cost of a generic drug.

~~(e)~~ (f) "Maximum allowable cost appeal pricing adjustment" means a retrospective positive payment adjustment made to a pharmacy by the pharmacy benefits plan or program or by the pharmacy benefit manager pursuant to an approved maximum allowable cost appeal request submitted by the same pharmacy to dispute the amount reimbursed for a drug based on the pharmacy benefit manager's listed maximum allowable cost price.

(g) "National average drug acquisition cost" or "NADAC" means a medication benchmark that represents the average retail price pharmacies pay to acquire prescription and over-the-counter drugs.

~~(f)~~ (h) "Participation contract" means any agreement between a pharmacy benefit manager and pharmacy for the provision and reimbursement of pharmacist services and any exhibits, attachments, amendments, or addendums to such agreement.

~~(g)~~ (i) "Pass-through pricing model" means a payment model used by a pharmacy benefit manager in which the payments made by the pharmacy ben-

efits plan or program to the pharmacy benefit manager for the covered outpatient drugs are:

(i) Equivalent to the payments the pharmacy benefit manager makes to a dispensing pharmacy or provider for such drugs, including any contracted professional dispensing fee between the pharmacy benefit manager and its network of pharmacies. Such dispensing fee would be paid if the pharmacy benefits plan or program was making the payments directly; and

(ii) Passed through in their entirety by the pharmacy benefits plan or program or by the pharmacy benefit manager to the pharmacy or provider that dispenses the drugs, and the payments are made in a manner that is not offset by any reconciliation.

~~(h)~~ (j) "Pharmacy benefit manager" means a person or entity doing business in this state that contracts with pharmacies on behalf of an insurer, third-party administrator, or managed care organization to administer prescription drug benefits to residents of this state.

~~(i)~~ (k) "Spread pricing" means the practice in which a pharmacy benefit manager charges a pharmacy benefits plan or program a different amount for pharmacist services than the amount the pharmacy benefit manager reimburses a pharmacy for such pharmacist services.

~~(j)~~ (l) "Usual and customary price" means the amount charged to cash customers for a pharmacist service exclusive of sales tax or other amounts claimed.

(2) A person may not perform, offer to perform, or advertise any pharmacy benefit management service in this state unless the person is registered as a pharmacy benefit manager with the department of insurance. A person may not utilize the services of another person as a pharmacy benefit manager if the person knows or has reason to know that the other person does not have a registration with the department. Such registration must occur annually no later than April 1 of each year and shall be on a form prescribed by the director. The department may utilize applicable sections of this title to administer registration as provided in this subsection.

(3) A pharmacy benefit manager shall not prohibit a pharmacist or retail pharmacy from providing a covered person information on the amount of the cost share for a prescription drug and the clinical efficacy of a more affordable alternative drug if one is available, and a pharmacy benefit manager may not penalize a pharmacist or retail pharmacy for disclosing such information to a covered person or for selling to a covered person a more affordable alternative if one is available.

(4) A pharmacy benefit manager shall not directly or indirectly charge a pharmacy benefits plan or program a different amount for a prescription drug's ingredient cost or dispensing fee than the amount the pharmacy benefit manager reimburses a pharmacy for the prescription drug's ingredient cost or dispensing fee where the pharmacy benefit manager retains the amount of any such difference.

(5) A pharmacy benefit manager shall apply the same utilization review, fees, copayments or cost-sharing, days allowance, and other conditions of a covered person when the covered person obtains a prescription drug from a pharmacy that is included in the pharmacy benefit manager's pharmacy net-

1 work, including mail-order pharmacies and the pharmacy benefit manager's
 2 owned, affiliated, or preferred pharmacies.

3 (6) A pharmacy benefit manager shall not:

4 (a) Reimburse a network pharmacy an amount less than the contract
 5 price between the pharmacy benefit manager and the insurer, third-party
 6 payer, or pharmacy services management organization; or

7 (b) Require or coerce a patient to use a pharmacy that is owned by or
 8 affiliated with the pharmacy benefit manager.

9 (7) A pharmacy benefit manager, a third-party payer, or a discount card
 10 processor shall not, directly or indirectly, charge or hold a pharmacy re-
 11 sponsible for any fee, including but not limited to the following:

12 (a) A fee for submission of a claim;

13 (b) Any other claim-related fee;

14 (c) A fee for enrollment or participation in a retail pharmacy network;

15 (d) A credentialing or recredentialing fee;

16 (e) A fee for the development or management of claims processing ser-
 17 vices or claims payment services; or

18 (f) A fee on remittance advice or a fee that is retroactive.

19 (8) All reimbursements to pharmacies shall be made through direct bank
 20 transfers, checks, or another payment method that does not incur any pro-
 21 cessing fees for the pharmacy. A check shall have a one-hundred-eighty (180)
 22 day expiration to deposit.

23 (9) A pharmacy benefit manager or third-party payer shall not prohibit
 24 a pharmacist or pharmacy from:

25 (a) Participating in a class action lawsuit;

26 (b) Disclosing to the plan sponsor or to the patient information re-
 27 garding the adjudicated reimbursement paid to the pharmacy if the phar-
 28 macist or pharmacy complies with the requirements of the federal health
 29 insurance portability and accountability act of 1996, 29 U.S.C. 1181,
 30 et seq.;

31 (c) Providing relevant information to a patient about the patient's
 32 prescription drug order, including but not limited to the cost and clin-
 33 ical efficacy of a more affordable alternative drug if one is available;

34 (d) Mailing or delivering a prescription drug to a patient as an ancil-
 35 lary service of a pharmacy if the practice is not prohibited by law; or

36 (e) Charging a shipping and handling fee to a patient who has asked that
 37 a prescription drug be mailed or delivered if the practice is not pro-
 38 hibited by law.

39 (10) A pharmacy benefit manager or third-party payer shall not:

40 (a) Require pharmacy accreditation standards or recertification re-
 41 quirements inconsistent with, more stringent than, or in addition to
 42 federal and state requirements for licensure as a pharmacy in this
 43 state; or

44 (b) Exclude a pharmacy from the pharmacy benefit manager's or third-
 45 party payer's network based solely on the pharmacy being newly opened or
 46 open for less than a defined period of time or because a license or loca-
 47 tion transfer occurs, unless there is pending investigation for fraud,
 48 waste, or abuse.

49 (11) A pharmacist or pharmacy that belongs to a pharmacy services admin-
 50 istrative organization shall be entitled to receive a copy of a contract be-

1 between the pharmacy services administrative organization and a pharmacy ben-
 2 efit manager or third-party payer on the pharmacy's or pharmacist's behalf.

3 (12) A pharmacy benefit manager or third-party payer shall provide a
 4 pharmacy or pharmacist with the processor control number, bank identifi-
 5 cation number, and group number for each pharmacy network established or
 6 administered by a pharmacy benefit manager or third-party payer to enable
 7 the pharmacy to make an informed contracting decision.

8 ~~(5)~~ (13) The pharmacy benefit manager shall pass along or return one
 9 hundred percent (100%) of any manufacturer rebate to a pharmacy benefits
 10 plan or program, including any payment, discount, incentive, fee, price
 11 concession, or other remuneration.

12 ~~(6)~~ (14) The pharmacy benefit manager shall provide full and complete
 13 disclosure of:

14 (a) The cost, price, and reimbursement of the prescription drug to each
 15 health plan, payer, and pharmacy with which the pharmacy benefit man-
 16 ager has a contract or agreement to provide pharmacy benefit management
 17 services;

18 (b) Each fee, markup, and discount charged or imposed by the pharmacy
 19 benefit manager to each health plan, payer, and pharmacy with which the
 20 pharmacy benefit manager has a contract or agreement for pharmacy bene-
 21 fit management services; or

22 (c) The aggregate amount of all remuneration the pharmacy benefit man-
 23 ager receives from a prescription drug manufacturer for a prescription
 24 drug, including any rebate, discount, administration fee, and any other
 25 payment or credit obtained or agreement for pharmacy benefit management
 26 services to a health plan or payer.

27 ~~(7)~~ (15) A pharmacy benefit manager using maximum allowable cost pric-
 28 ing may place a drug on a maximum allowable cost list if the pharmacy benefit
 29 manager does the following:

30 (a) Ensures that the drug:

31 (i) 1. Is listed as A-rated or B-rated in the most recent ver-
 32 sion of the United States food and drug administration's ap-
 33 proved drug products with therapeutic equivalence evalua-
 34 tions, also known as the "orange book"; or

35 2. Has an NR or NA rating or a similar rating by a nationally
 36 recognized reference; and

37 (ii) Is available for purchase by pharmacies in the state from na-
 38 tional or regional wholesalers and is not obsolete;

39 (b) Provides to a network pharmacy, at the time a contract is entered
 40 into or renewed with the network pharmacy, the sources used to determine
 41 the maximum allowable cost pricing for the maximum allowable cost list
 42 specific to that provider;

43 (c) Reviews and updates maximum allowable cost price information at
 44 least once every seven (7) business days to reflect any modification of
 45 maximum allowable cost pricing;

46 (d) Establishes a process for eliminating products from the maximum al-
 47 lowable cost list or modifying maximum allowable cost prices in a timely
 48 manner to remain consistent with pricing changes and product availabil-
 49 ity in the marketplace;

(e) Establishes a process by which a network pharmacy, or a network pharmacy's contracting agent, may appeal the reimbursement for a generic drug no later than thirty (30) days after such reimbursement is made; and

(f) Provides a process for each of its network pharmacies to readily access the maximum allowable cost list specific to that provider.

(16) A pharmacy benefit manager or third-party payer shall not make or allow any reduction in payment for pharmacy services by a pharmacy benefit manager or third-party payer or directly or indirectly reduce a payment for pharmacy services under a reconciliation process to an effective rate of reimbursement, including generic effective rates, brand effective rates, direct and indirect remuneration fees, or any other reduction or aggregate reduction of payments.

~~(8)~~ (17) No pharmacy benefit manager may retroactively deny or reduce a claim for reimbursement of the cost of services after the claim has been adjudicated by the pharmacy benefit manager unless:

(a) The adjudicated claim was submitted fraudulently or improperly; or

(b) The pharmacy benefit manager's payment on the adjudicated claim was incorrect because the pharmacy or pharmacist had already been paid for the services.

~~(9)~~ (18) If the director finds a pharmacy benefit manager has violated this section or any provision of title 41, Idaho Code, then the director may subject the pharmacy benefit manager to any or all of the actions, penalties, and remedies referenced in sections 41-117, 41-1016, and 41-1026, Idaho Code.

~~(10)~~ (19) (a) No later than January 1, 2025, and each year thereafter, each licensed pharmacy benefit manager shall report to the director of the department of insurance the following information:

(i) The aggregate amount of the difference between the amount the pharmacy benefit manager paid each pharmacy on behalf of the health plan for prescription drugs; and

(ii) If at any time during the reporting year the pharmacy benefit manager moved or reassigned a prescription drug to a formulary tier that has a higher cost, higher copayment, higher coinsurance, higher deductible to a consumer, or lower reimbursement to a pharmacy, an explanation of the reason why the drug was moved or reassigned, including whether the move or reassignment was determined or requested by a prescription drug manufacturer or other entity.

(b) Any pharmacy benefit manager that owns, controls, or is affiliated with a pharmacy shall also report any difference in reimbursement rates or practices, direct and indirect remuneration fees or other price concessions, and clawbacks between a pharmacy that is owned, controlled, or affiliated with the pharmacy benefit manager and any other pharmacy.

~~(11)~~ (20) In addition to any other requirements in this title, all contractual arrangements executed, amended, adjusted, or renewed between a pharmacy benefit manager and a pharmacy benefits plan or program must include, in substantial form, requirements, to the extent allowable by law, to:

(a) Use a pass-through pricing model;

1 (b) Exclude terms that allow for the direct or indirect engagement in
2 the practice of spread pricing;

3 (c) Ensure that funds received in relation to providing services for a
4 pharmacy benefits plan or program or a pharmacy are used or distributed
5 only pursuant to the pharmacy benefit manager's contract with the phar-
6 macy benefits plan or program or with the pharmacy or as otherwise re-
7 quired by applicable law;

8 (d) Require the pharmacy benefit manager to pass one hundred percent
9 (100%) of all prescription drug manufacturer rebates, including non-
10 resident prescription drug manufacturer rebates, received to the phar-
11 macy benefits plan or program, if the contractual arrangement delegates
12 the negotiation of rebates to the pharmacy benefit manager, for the
13 sole purpose of offsetting defined cost-sharing and reducing premiums
14 of covered persons. Rebates include any payment, discount, incentive,
15 fee, price concession, or other remuneration. Any excess rebate rev-
16 enue after the pharmacy benefit manager and the pharmacy benefits plan
17 or program have taken all actions required pursuant to this section must
18 be used for the sole purpose of offsetting copayments and deductibles of
19 covered persons;

20 (e) Include network adequacy requirements that meet or exceed medicare
21 part D program standards for convenient access to the network pharma-
22 cies and that:

23 (i) Do not limit a network to solely include affiliated pharma-
24 cies;

25 (ii) Do not require a covered person to receive a prescrip-
26 tion drug by United States mail, common carrier, local courier,
27 third-party company or delivery service, or pharmacy direct de-
28 livery unless the prescription drug cannot be acquired at any
29 retail pharmacy in the pharmacy benefit manager's network for
30 the covered person's pharmacy benefits plan or program. The
31 provisions of this subparagraph do not prohibit a pharmacy bene-
32 fit manager from operating mail order or delivery programs on an
33 opt-in basis at the sole discretion of a covered person, provided
34 that the covered person is not penalized through the imposition
35 of any additional retail cost-sharing obligations or a lower al-
36 lowed-quantity limit for choosing not to select the mail order or
37 delivery programs;

38 (iii) For the in-person administration of covered prescription
39 drugs, prohibit requiring a covered person to receive pharmacist
40 services from an affiliated pharmacy or an affiliated health care
41 provider; and

42 (iv) Prohibit offering or implementing pharmacy networks that re-
43 quire or provide a promotional item or an incentive to a covered
44 person to use an affiliated pharmacy or an affiliated health care
45 provider for the in-person administration of covered prescription
46 drugs or advertising, marketing, or promoting an affiliated phar-
47 macy to covered persons. Provided, however, a pharmacy benefit
48 manager may include an affiliated pharmacy in communications to
49 covered persons regarding network pharmacies and prices as long as
50 the pharmacy benefit manager includes information, such as links

1 to all nonaffiliated network pharmacies, in such communications
2 and that the information provided is accurate and of equal promi-
3 nence. The provisions of this subparagraph may not be construed to
4 prohibit a pharmacy benefit manager from entering into an agree-
5 ment with an affiliated pharmacy to provide pharmacist services to
6 covered persons;

7 (f) Prohibit a pharmacy benefit manager from conditioning participa-
8 tion in one (1) pharmacy network based on participation in any other
9 pharmacy network or from penalizing a pharmacy for exercising its pre-
10 rogative not to participate in a specific pharmacy network;

11 (g) Prohibit a pharmacy benefit manager from instituting a network
12 that requires a pharmacy to meet accreditation standards inconsistent
13 with or more stringent than applicable federal and state requirements
14 for licensure and operation as a pharmacy in this state. However, a
15 pharmacy benefit manager may specify additional specialty networks
16 that require enhanced standards related to safety and competency
17 necessary to meet the United States food and drug administration's
18 limited distribution requirements for dispensing any drug that, on a
19 drug-by-drug basis, requires extraordinary special handling, provider
20 coordination, or clinical care or monitoring when such extraordinary
21 requirements cannot be met by a retail pharmacy. For purposes of this
22 paragraph, drugs requiring extraordinary special handling are limited
23 to drugs that are subject to a risk evaluation and mitigation strategy
24 approved by the United States food and drug administration and that:

25 (i) Require special certification of a health care provider to
26 prescribe, receive, dispense, or administer; or

27 (ii) Require special handling due to the molecular complexity
28 or cytotoxic properties of the biologic or biosimilar product or
29 drug. For participation in a specialty network, a pharmacy ben-
30 efit manager may not require a pharmacy to meet requirements for
31 participation beyond those necessary to demonstrate the phar-
32 macy's ability to dispense the drug in accordance with the United
33 States food and drug administration's approved manufacturer la-
34 beling;

35 (h) At a minimum, require the pharmacy benefit manager or pharmacy ben-
36 efits plan or program to, upon revising its formulary of covered pre-
37 scription drugs during a plan year, provide a ninety (90) day continu-
38 ity-of-care period in which the covered prescription drug that is being
39 revised from the formulary continues to be provided at the same cost for
40 the patient for a period of ninety (90) days. The ninety (90) day conti-
41 nuity-of-care period commences upon notification to the patient. This
42 requirement does not apply if the covered prescription drug:

43 (i) Has been approved and made available over the counter by the
44 United States food and drug administration and has entered the
45 commercial market as such;

46 (ii) Has been removed or withdrawn from the commercial market by
47 the manufacturer;

48 (iii) Is subject to an involuntary recall by state or federal au-
49 thorities and is no longer available on the commercial market; or

1 (iv) Has a generic, biosimilar, or interchangeable biologic ap-
 2 proved by the United States food and drug administration;
 3 (i) Require that in-network pharmacies receive dispensing fees that
 4 reasonably cover the costs of dispensing medications pursuant to sec-
 5 tion 41-349A, Idaho Code; and
 6 (j) Prohibit a pharmacy benefit manager from directly or indirectly
 7 charging or holding a pharmacist or pharmacy responsible for a fee for
 8 any step of or component or mechanism related to the claim adjudication
 9 process, including:
 10 (i) The adjudication of a pharmacy benefit claim;
 11 (ii) The processing or transmission of a pharmacy benefit claim;
 12 (iii) The development or management of a claim processing or adju-
 13 dication network; or
 14 (iv) Participation in a claim processing or adjudication network.
 15 ~~(12)~~ (21) The requirements of subsection ~~(11)~~ (20) of this section shall
 16 not apply to specialty drugs. For the purposes of this section, "specialty
 17 drug" means:
 18 (a) A drug that is subject to restricted distribution by the United
 19 States food and drug administration; or
 20 (b) A drug that requires special handling, provider coordination, or
 21 patient education that a retail pharmacy cannot provide.
 22 ~~(13)~~ (22) In addition to other requirements in this title, a partici-
 23 pation contract executed, amended, adjusted, or renewed between a pharmacy
 24 benefit manager and one (1) or more pharmacies or pharmacists must include,
 25 in substantial form, to the extent allowable by law, terms that ensure com-
 26 pliance with the provisions of this subsection.
 27 (a) The pharmacy benefit manager shall provide a reasonable adminis-
 28 trative appeal procedure to allow a pharmacy or pharmacist to challenge
 29 the maximum allowable cost pricing information and the reimbursement
 30 made under the maximum allowable cost as defined in subsection (1) (d)
 31 of this section for a specific drug as being below the acquisition cost
 32 available to the challenging pharmacy or pharmacist.
 33 (b) The administrative appeal procedure must include a telephone num-
 34 ber and email address, or a website, for the purpose of submitting the
 35 administrative appeal. The appeal may be submitted by the pharmacy or
 36 an agent of the pharmacy directly to the pharmacy benefit manager or
 37 through a pharmacy service administration organization. The pharmacy
 38 or pharmacist must be given at least thirty (30) business days after
 39 a maximum allowable cost update or after an adjudication for an elec-
 40 tronic claim or reimbursement for a nonelectronic claim to file the
 41 administrative appeal.
 42 (c) The pharmacy benefit manager must respond to the administrative ap-
 43 peal within thirty (30) business days after receipt of the appeal.
 44 (i) If the appeal is upheld, the pharmacy benefit manager must:
 45 1. Update the maximum allowable cost pricing information to
 46 at least the acquisition cost available to the pharmacy;
 47 2. Permit the pharmacy or pharmacist to reverse and rebill
 48 the claim in question;
 49 3. Provide to the pharmacy or pharmacist the national drug
 50 code on which the increase or change is based; and

1 4. Make the increase or change effective for each similarly
2 situated pharmacy or pharmacist who is subject to the appli-
3 cable maximum allowable cost pricing information; or
4 (ii) If the appeal is denied, the pharmacy benefit manager must
5 provide to the pharmacy or pharmacist the national drug code and
6 the name of the national or regional pharmaceutical wholesalers
7 operating in this state that have the drug currently in stock at a
8 price below the maximum allowable cost pricing information.
9 (d) Every ninety (90) days, a pharmacy benefit manager shall report to
10 the department the total number of appeals received and denied in the
11 preceding ninety (90) day period, with an explanation or reason for each
12 denial, for each specific drug for which an appeal was submitted pur-
13 suant to this subsection.
14 ~~(14)~~ (23) In addition to other prohibitions in this section, a pharmacy
15 benefit manager may not do any of the following:
16 (a) Prohibit, restrict, or penalize in any way a pharmacy or pharmacist
17 from disclosing to any person any information that the pharmacy or phar-
18 macist deems appropriate, including but not limited to information re-
19 garding any of the following:
20 (i) The nature of treatment, risks, or alternatives thereto;
21 (ii) The availability of alternate treatment, consultations, or
22 tests;
23 (iii) The decision of utilization reviewers or similar persons to
24 authorize or deny pharmacist services;
25 (iv) The process used to authorize or deny pharmacist services or
26 benefits;
27 (v) Information on financial incentives and structures used by
28 the pharmacy benefits plan or program;
29 (vi) Information that may reduce the costs of pharmacist ser-
30 vices;
31 (vii) Whether the cost-sharing obligation exceeds the retail
32 price for a covered prescription drug and the availability of a
33 more affordable alternative drug;
34 (viii) A decision by the pharmacy to refuse to accept pharmacy ben-
35 efit manager payment for the dispensing of an individual prescrip-
36 tion on the basis of an aggregate pharmacy benefit manager payment
37 of less than the pharmacy's costs to provide the service; or
38 (ix) The financial details of a prescription claim;
39 (b) Prohibit, restrict, or penalize in any way a pharmacy or pharma-
40 cist from disclosing information to the department, law enforcement, or
41 state and federal governmental officials, provided that the recipient
42 of the information represents that it has the authority, to the extent
43 provided by state or federal law, to maintain proprietary information
44 as confidential and before disclosure of information designated as con-
45 fidential, the pharmacist or pharmacy marks as confidential any docu-
46 ment in which the information appears or requests confidential treat-
47 ment for any oral communication of the information;
48 (c) Communicate at the point-of-sale, or otherwise require, a cost-
49 sharing obligation for the covered person in an amount that exceeds the
50 lesser of:

1 (i) The applicable cost-sharing amount under the applicable
 2 pharmacy benefits plan or program; or
 3 (ii) The amount that will be retained by the pharmacy;
 4 (d) Transfer or share records relative to prescription information
 5 containing patient-identifiable or prescriber-identifiable data
 6 to an affiliated pharmacy for any commercial purpose other than the
 7 limited purposes of facilitating pharmacy reimbursement, formulary
 8 compliance, or utilization review on behalf of the applicable pharmacy
 9 benefits plan or program;
 10 (e) Fail to make any payment due to a pharmacy for an adjudicated claim
 11 with a date of service before the effective date of a pharmacy's ter-
 12 mination from a pharmacy benefit network, unless payments are withheld
 13 because of fraud, waste, or abuse on the part of the pharmacy or except
 14 as otherwise required by law; or
 15 (f) Terminate the contract of, penalize, or disadvantage a pharmacist
 16 or pharmacy solely due to a pharmacist or pharmacy:
 17 (i) Disclosing information about pharmacy benefit manager prac-
 18 tices in accordance with this section;
 19 (ii) Exercising any of its prerogatives pursuant to this section;
 20 or
 21 (iii) Sharing any portion, or all, of the pharmacy benefit manager
 22 contract with the department of insurance pursuant to a complaint
 23 or a query regarding whether the contract is in compliance with the
 24 provisions of this section.
 25 ~~(15)~~ (24) In complying with the requirements of this section, a pharmacy
 26 benefit manager or its agents, and the director or the director's agents,
 27 shall not directly or indirectly publish or otherwise disclose any infor-
 28 mation reported to the director under this section that would reveal: the
 29 identity of a specific pharmacy benefits plan, program, or pharmaceutical
 30 manufacturer; the prices charged for a specific drug or class of drugs; the
 31 amount of any rebates provided for a specific drug or class of drugs or the
 32 pharmaceutical manufacturer; or information that would otherwise have the
 33 potential to compromise the financial, competitive, or proprietary nature
 34 of such information. Any such information shall be protected from disclo-
 35 sure as confidential and proprietary and shall not be regarded as a public
 36 record pursuant to section 74-101, Idaho Code. A pharmacy benefit manager
 37 shall impose the confidentiality protections and requirements of this sec-
 38 tion on any agent or downstream third party that performs health care or ad-
 39 ministrative services on behalf of the pharmacy benefit manager that may re-
 40 ceive or have access to such information, and the director shall impose the
 41 confidentiality protections and requirements of this section on any agent
 42 or downstream third party directly or indirectly involved in the administra-
 43 tion of this section that may receive or have access to such information.

44 SECTION 3. An emergency existing therefor, which emergency is hereby
 45 declared to exist, this act shall be in full force and effect on and after its
 46 passage and approval.